DPP — DEPARTMENTAL POLICY AND PROCEDURE

TITLE:	CARE OF INFANT OF DIABETIC MOTHER
DEPARTMENT:	NEONATAL INTENSIVE CARE UNIT
APPLIES TO:	NICU PHYSICIANS AND NURSES

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1. PURPOSE

1.1. To provide guidelines to health care providers on how to care for infants of diabetic mothers.

1.2. To ensure proper assessment of the infant of diabetic mother for prompt treatment and prevention of its complications.

2. DEFINITION & ABBREVIATION

2.1. **IDM (Infant of Diabetic Mother)** – is a baby born to the mother who has diabetes.

2.2. **IV** – Intravenous

2.3. **PPE** – Personal Protective Equipment

2.4. **NICU** – Neonatal Intensive Care Unit

3. RESPONSIBILITY

3.1. **Physicians (Consultant, Specialists and Residents)**

3.1.1. Responsible to assess and evaluate the infant of diabetic mother and providing written orders in the medical file.

3.2. **Nurses (Head Nurse, Charge Nurse and Staff Nurse)**

3.2.1. Responsible to assess and monitor the patient with signs and symptoms associated with IDM.

3.2.2. Carry out the procedures ordered to manage complications of IDM.

4. POLICY

4.1. Newborn infants of diabetic mother who has controlled blood glucose and who does not present any complications of IDM after careful observation in Nursery will be roomed-in with the mother to promote early breastfeeding.

4.2. Infants who still present an uncontrolled blood glucose level after careful evaluation will be subjected for admission to Neonatal Intensive Care Unit for further management.

4.3. Frequency of infant's blood glucose monitoring should be according to the newborn's clinical condition and upon the written order of the physician.

4.4. **Assessment Criteria of Infant of Diabetic Mother**

4.4.1. **Symptomatic Infants**

4.4.1.1. Presence of jitteriness

4.4.1.2. Cyanosis

4.4.1.3. Seizures, eye rolling

4.4.1.4. Apneic episodes

4.4.1.5. Tachypnea

4.4.1.6. Weak or high-pitched cry

4.4.1.7. Lethargy or coma

4.4.2. **Asymptomatic Infants**

4.4.2.1. **Birth to 4 hours of life**

4.4.2.1.1. After initial feed before 1 hour of age, the first glucose screen is initiated within half an hour. If below 25 mg/dl and patient is still asymptomatic, NB should be re-feed, and checked within an hour.

4.4.2.1.2. In case blood glucose is still less than 25 mg/dl, intravenous glucose therapy should be initiated.

4.4.2.1.3. If the blood glucose lies between 25-40 mg/dl, the infant should be re-feed; will necessitate admission to NICU for Intravenous fluid therapy.

4.4.2.1.4. Subsequent concentrations lower than 25 - 40 mg/dl, the infant should be re-feed; will necessitate admission to NICU for Intravenous fluid therapy.

4.4.2.2. **4-12 hours of life**

4.4.2.2.1. The infant should receive feeds every 2-3 hours, and his blood glucose must be checked prior to every feed.

4.4.2.2.2. The target blood glucose concentrations is greater than 45 mg/dl before each feeding but in the event blood glucose is below 35 mg/dl and the patient is still asymptomatic, the infant should be reefed, and checked within an hour.

4.4.2.2.3. In case blood glucose is still less than 35 mg/dl, IV glucose therapy should be initiated.

4.4.2.2.4. If blood glucose lies between 35 and 45 mg/dl, the infant should be re-feed.

4.4.2.2.5. Subsequent concentrations lower than 35 mg/dl after attempts to re-feed, will necessitate admission to NICU for intravenous fluid therapy.

4.5. All IDM's with respiratory symptoms should undergo a chest X-ray and an echocardiogram to verify the presence or absence of an anatomic heart disease.

4.6. Good management of temperature control must be observed to further avoid potential IDM complication such as neonatal hypoglycemia.

4.7. Management of respiratory distress in IDM will be managed accordingly such as administration of oxygen or other ventilator support.

4.8. Laboratory investigations are indicated to screen for hypocalcaemia, hypomagnesaemia, thrombocytopenia and polycythemia or as directed by the physician.

5. PROCEDURE

5.1. The physician and nurse will assess the infant of the diabetic mother.

5.2. If the infant is well and does not present any complication of IDM, room-in the infant with the mother for breastfeeding as ordered.

5.3. Perform continuous re-assessment of the patient after being room-in with the mother.

5.4. Is infant is unwell with presenting signs and symptoms of IDM, the physician will provide a written order of the procedure to be done to the patient in the medical file.

5.5. The nurse will verify the procedure ordered by the physician to manage complications of IDM.

5.6. The nurse will identify the patient correctly, perform hand hygiene and wear PPE according to policy.

5.7. The nurse will carry out the procedures to be done as ordered (e.g. feeding, glucose monitoring, oxygen therapy).

5.8. Document all the procedure done to patient in the medical file.

6. MEASUREMENT

6.1. 100% of the staff is knowledgeable and adhere to this policy and procedures.

7. REFERENCES /CROSS- REFERENCES

7.1. PMSH-DPP-NICU-0018(v3): Care of Infant of Diabetic Mother, 28/05/2023, p. 1-4

7.2. CBAHI Standards 3rd edition, 01/01/2016, NICU.10 p.126-127

7.3. https://www.uptodate.com/contents/infant-of-a-diabetic-mother

7.4. https://www.google.com.sa/search/q-infant-of-the-diabetic/mother/medscape

8. ATTACHMENT

8.1. None

9. REVISION CLAUSE

9.1. Changes to the hospital policy and procedure template based from the Al-Baha Health Cluster logo, font face and design.